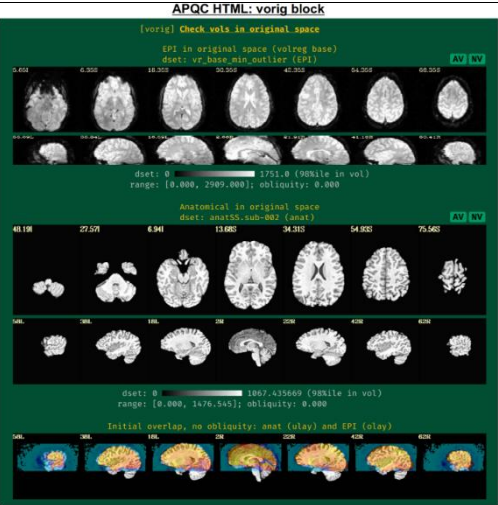
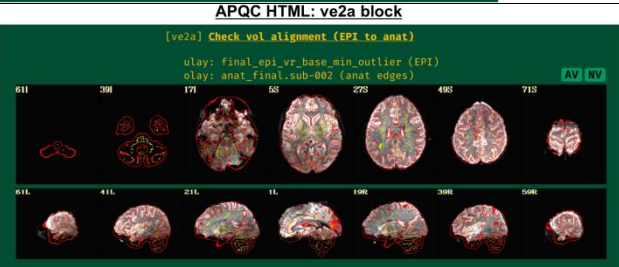
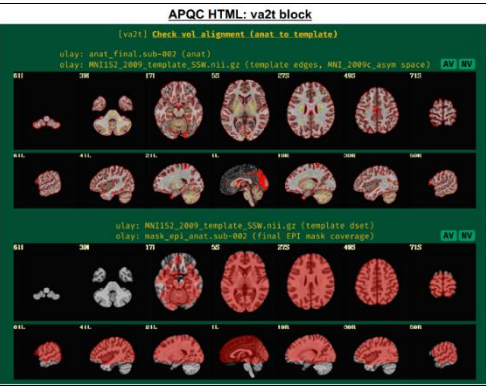


Stage	Program	Block	Description	Qualitative	Quantitative	Output
Raw data evaluation			- getting to know your data, GTKYD	Yes	Yes	
	AFNI GUI		- visualize data images, time series graphs, and more	- align: Check alignment (or registration) features. - graph: View the time series plots of one or more voxels. - instacorr: Flag peculiar spatio-temporal patterns in the time series data. - other: Any other feature(s) using the afni and/or suma GUIs, plugins, etc.		
	gtykd_check.py		- summarize header and data (min/max) properties - verify desired properties for one or more datasets - evaluate consistency of properties across group		- header-info: Matrix size, orientation, voxel dimensions, datum type, NIFTI qform_code, NIFTI sform_code - data-info: Number of runs, minimum value, maximum value. - EPI header-info: TR, number of time points, slice timing.	
Prepressing – Structure T1*	sswaper2		- bias field correction & grad unwarp	- Field of view (FOV). - Distortion. - High resolution and contrast. - Alignment is good: tissue boundaries and sulcal and gyral patterns appear to be well-aligned.		In the T1 results folder, AM*.jpg, MA*.jpg, QC_anatQQ*.jpg and QC_anatSS*.jpg are the images that we care most. In the ssw_align_hist.* folder you will see the step-by-step images generated in the process, not very necessary.
Preprocessing - APQC HTML	afni_proc.py or ap_run_simple_res t.tcsh: gen_epi_review.py (@epi_review), gen_ss_review_scripts.py (@ss_review_basic, @ss_review_driver, and @ss_review_driver_commands), apqc_make_tcsh.py, apqc_make_html.py , run qc *.tcsh		- evaluate systematic images and quantities (alignment, regression, etc.) - run interactive data exploration in GUIs - save QC ratings and commentsGroup summary tables (of afni_proc.py and more)	Yes	Yes	QC_\${subj} directory: out.ss_review.\$subj.txt, out.ss_review_uvars.json <pre>{ "afni_package": "Linux_cmake", "afni_ver": "AFNI_99.99.99", "align_anat": "091_al_keep+orig.HEAD", "censor_dset": "censor_091_combined.2.1D", "copy_anat": "anatSS_s091+orig.HEAD", "cormat_warn_dset": "out.cormat_warn.txt", "df_info_dset": "out.df_info.txt", "enorm_dset": "motion_091_enorm.1D", "errts_dset": "errts.091_REML+tlrc.HEAD", "final_anat": "anat_final.091+tlrc.HEAD", "final_epi_dset": "final_epi_vr_base_min_outlier+tlrc.HEAD", "final_view": "tlrc", "flip_check_dset": "aea_checkflip_results.txt", "flip_guess": "NO_FLIP", "gcor_dset": "out.gcor.1D", "have_radcor_dirs": "yes", "mask_anat_tmpl_corr_dset": "out.mask_at_dice.txt", "mask_corr_dset": "out.mask_ae_dice.txt", "mask_dset": "mask_epi_anat.091+tlrc.HEAD", "max_4095_warn_dset": "out.4095_warn.txt", "mb_level": 1, "mot_limit": 0.3, "motion_dset": "dfibs_well.1D", "num_TRs_above_mot_limit": 5, "average_motion_per_TR": 0.103108, "average_censored_motion": 0.0993947, "max_motion_displacement": 1.64944, "max_censored_displacement": 1.63683, "outlier_limit": 0.05, "average_outlier_freq_TR": 0.00619577, "subject_ID": 091, "AFNI_version": "AFNI_99.99.99", "AFNI_package": "Linux_cmake", "TR": 1.0, "TRs_removed_per_run": 0, "multiband_level": 1, "slice_timing_pattern": "simult", "num_stim_classes_provided": 3, "final_anatomy_dset": "anat_final.091+tlrc.HEAD", "final_stats_dset": "stats.091_REML+tlrc.HEAD", "orig_voxel_counts": [102, 102, 64], "orig_voxel_resolution": [2.509800, 2.509800, 2.499786], "orig_volume_center": [-18.062111, -15.281898, -11.606758], "final_voxel_resolution": [2.500000, 2.500000, 2.500000]</pre>

	gen_ss_review_scripts.py	Vorig	- EPI and anatomical FOV coverage, distortion, tissue contrast, brightness inhomogeneity. - Initial EPI-anatomical overlap	- The entire brain is in the FOV (no missing region) - No severe brightness inhomogeneity or distortion - appropriate tissue contrast - No flip - Initial EPI-anatomical overlap (alignment, bottom row) is not far away from each other	- The grayscale colorbar does not contain negative values (indicating potential issues with data acquisition or conversion) and that the maximum value does not exceed 4095 (suggesting saturation of the 12-bit intensity signal).	
		ve2a	- EPI-anatomical alignment - EPI coverage, distortion	- Good alignment: similar internal structures in the same location, such as ventricle edges, tissue boundaries, and gyral/sulcal patterns. In judging EPI-anatomical alignment, interior structures matter most and CSF should be ignored. - Poor: low or nonstandard tissue contrast. - Good signal coverage (any missing region or distortion in functional ROI?)		
		va2t	- anatomical-template alignment - Final EPI coverage in template: the EPI mask¹ (calculated as the intersection of the anatomical and EPI data) is aligned with the template.	- Good alignment (e.g., from sulcal, gyral and ventricle patterns). - EPI coverage is relatively strong or potentially missing.		

¹ Mask-overlap: Estimated coverage of usable fMRI signal (typically intersected with the subject anatomical).

				non-GM (gray matter) tissues can be scanner issues, motion-related artifacts, or having too few DFs left in the time series. - ROIs in template: enough voxels.	- TSNR_final-artifact: Non-physiological patterns of TSNR magnitude, particularly dropout (e.g., vertical bands/stripes). - ROI shape and TSNR stats table: make sure the ROI has enough signal and voxels if the whole TSNR is low. - Nvox, total number of voxels in ROI (warnings if quite small) - Nzer, number of zerovalued voxels (warnings as the fraction Nzer/Nvox increases) - Dvox, the maximum depth of this ROI, counted in voxels (warnings as this decreases) - Tmin, T25%, Tmed, T75%, Tmax, which are the min, max, and quartile values of TSNR (warnings as T75%—T25%)/Tmed increases)	APQC HTML: regr block (part 3) A) ROI TSNR and shape table, with automatic highlighting of potential issues ROI shape and TSNR stats (tsnr_stats_regress/stats_auto_MNI_2009c_asym.txt) dset: MNI_2009c_asym (ROI import MNI_2009c_asym_resam), Nroi: 12 <table><tr><th>ROI</th><th>Nvox</th><th>Nzer</th><th>Dvox</th><th>Tmin</th><th>T25%</th><th>Tmed</th><th>T75%</th><th>Tmax</th><th>X</th><th>Y</th><th>Z</th><th>ROI_name</th></tr><tr><td>10</td><td>468</td><td>0</td><td>2.2</td><td>44</td><td>143</td><td>206</td><td>245</td><td>328</td><td>5.0</td><td>91.8</td><td>0.5</td><td>SYAF-LH_VisCent_Striate_1</td></tr><tr><td>11</td><td>1071</td><td>4</td><td>2.8</td><td>29</td><td>182</td><td>237</td><td>274</td><td>452</td><td>42.5</td><td>16.8</td><td>55.5</td><td>SYAF-LH_SomMotA_1</td></tr><tr><td>12</td><td>794</td><td>0</td><td>2.4</td><td>65</td><td>121</td><td>159</td><td>200</td><td>305</td><td>7.5</td><td>-28.8</td><td>33.0</td><td>SYAF-LH_SalVentAttnB_PFCmp_1</td></tr><tr><td>13</td><td>1337</td><td>5</td><td>3.0</td><td>5</td><td>88</td><td>80</td><td>82</td><td>325</td><td>15.0</td><td>-23.2</td><td>-22.0</td><td>SYAF-LH_LimbicB_OFC_1</td></tr><tr><td>14</td><td>1971</td><td>3</td><td>0</td><td>0</td><td>50</td><td>73</td><td>105</td><td>185</td><td>27.5</td><td>11.8</td><td>-29.5</td><td>SYAF-LH_LimbicA_TempPole_1</td></tr><tr><td>20</td><td>839</td><td>15</td><td>2.2</td><td>9</td><td>139</td><td>179</td><td>212</td><td>339</td><td>-10.0</td><td>66.8</td><td>19.5</td><td>GHCP-R_Primary_Visual_Cortex</td></tr><tr><td>21</td><td>282</td><td>15</td><td>1.4</td><td>67</td><td>171</td><td>220</td><td>265</td><td>405</td><td>-7.5</td><td>34.2</td><td>78.0</td><td>GHCP-R_Primary_Sensory_Cortex</td></tr><tr><td>22</td><td>92</td><td>0</td><td>1.7</td><td>108</td><td>174</td><td>203</td><td>240</td><td>314</td><td>-5.0</td><td>-25.8</td><td>28.0</td><td>GHCP-R_Anterior_24_prime</td></tr><tr><td>23</td><td>342</td><td>0</td><td>2.2</td><td>8</td><td>18</td><td>21</td><td>25</td><td>93</td><td>-10.0</td><td>-35.8</td><td>-24.5</td><td>GHCP-R_Orbital_Frontal_Complex</td></tr><tr><td>24</td><td>660</td><td>3</td><td>2.2</td><td>15</td><td>42</td><td>62</td><td>110</td><td>271</td><td>-45.0</td><td>-15.8</td><td>-34.5</td><td>GHCP-R_Area_TG_dorsal</td></tr><tr><td>30</td><td>180</td><td>11</td><td>2.2</td><td>10</td><td>27</td><td>47</td><td>78</td><td>149</td><td>-22.5</td><td>-8.8</td><td>-24.5</td><td>JULI-RH_Amygdala</td></tr><tr><td>31</td><td>34</td><td>0</td><td>1.0</td><td>62</td><td>166</td><td>195</td><td>217</td><td>237</td><td>-5.0</td><td>-8.2</td><td>-4.5</td><td>JULI-RH_Accumbens</td></tr></table> B) ROIs in default MNI TSNR/shape table (APQC_atlas_MNI_2009c_asym.nii.gz) Left hemisphere Right hemisphere SYAF-LH_SalVentAttnB_PFCmp_1 SYAF-LH_SomMotA_1 SYAF-LH_VisCent_Striate_1 SYAF-LH_LimbicB_OFC_1 SYAF-LH_LimbicA_TempPole_1 GHCP-R_Primary_Visual_Cortex GHCP-R_Primary_Sensory_Cortex GHCP-R_Anterior_24_prime GHCP-R_Orbital_Frontal_Complex GHCP-R_Area_TG_dorsal Atlas abbreviations (ROI names from each) SYAF = Schaefer-Yeo, AFNI-refined version GHCP = Glasser et al. HCP JULI = Julich-Brain JULI-RH_Accumbens JULI-RH_Amygdala	ROI	Nvox	Nzer	Dvox	Tmin	T25%	Tmed	T75%	Tmax	X	Y	Z	ROI_name	10	468	0	2.2	44	143	206	245	328	5.0	91.8	0.5	SYAF-LH_VisCent_Striate_1	11	1071	4	2.8	29	182	237	274	452	42.5	16.8	55.5	SYAF-LH_SomMotA_1	12	794	0	2.4	65	121	159	200	305	7.5	-28.8	33.0	SYAF-LH_SalVentAttnB_PFCmp_1	13	1337	5	3.0	5	88	80	82	325	15.0	-23.2	-22.0	SYAF-LH_LimbicB_OFC_1	14	1971	3	0	0	50	73	105	185	27.5	11.8	-29.5	SYAF-LH_LimbicA_TempPole_1	20	839	15	2.2	9	139	179	212	339	-10.0	66.8	19.5	GHCP-R_Primary_Visual_Cortex	21	282	15	1.4	67	171	220	265	405	-7.5	34.2	78.0	GHCP-R_Primary_Sensory_Cortex	22	92	0	1.7	108	174	203	240	314	-5.0	-25.8	28.0	GHCP-R_Anterior_24_prime	23	342	0	2.2	8	18	21	25	93	-10.0	-35.8	-24.5	GHCP-R_Orbital_Frontal_Complex	24	660	3	2.2	15	42	62	110	271	-45.0	-15.8	-34.5	GHCP-R_Area_TG_dorsal	30	180	11	2.2	10	27	47	78	149	-22.5	-8.8	-24.5	JULI-RH_Amygdala	31	34	0	1.0	62	166	195	217	237	-5.0	-8.2	-4.5	JULI-RH_Accumbens
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	@radial_correlate	radcor	- local correlation patterns across the brain, InstaCorr of EPI at different stages - @radial_correlate calculates radial correlation (radcor) of each voxel in a dataset; that is, the correlation with a Gaussian-weighted average of the voxels in a large radius (full width at half maximum (FWHM) of 20 mm, by default) around it to detect certain kinds of coil artifacts or motion, which can produce large, highly correlated patches across the brain.	- As long as you indicate in afni_proc.py, AFNI will make radcor at each stage (e.g. after processing a head movement). Make sure to check the radcor values both before and after volume registration to reduce the effects of subject motion. - Motion: high correlation following the edge of the brain or EPI FOV. - Bad coil: anterior and right octant of the brain is extremely highly correlated. - In regression stage, seeing high radcor values constrained within GM can be a positive indication that both motion effects are reduced (because those would likely produce correlations across tissue boundaries) and that task response is high (because that would likely increase correlations across functionally related GM).		APQC HTML: radcor block Subset of images for sub-002 [radcor] Check extent of local correlation @radial_correlate check for data dir: radcor.pb00.tcatt olay: radcor.20.r01.corr IC GV AV NV @radial_correlate check for data dir: radcor.pb02.volreg olay: radcor.20.r01.corr IC GV AV NV @radial_correlate check for data dir: radcor.pb05.regress olay: radcor.20.d02.ertrts.corr IC GV AV NV olay: -0.7 thr: 0.4 (alpha on) 0.7 (ulay is MNI152_2009_template_55W.nii.gz) Example of radcor highlighting scanner artifact (separate demo subject) [radcor] Check extent of local correlation @radial_correlate check for data dir: radcor.pb05.regress olay: radcor.20.d02.ertrts.corr IC GV AV NV olay: -0.7 thr: 0.4 (alpha on) 0.7 (ulay is anat_final.sub-620+orig.HEAD)																																																																																																																																																																									

Reference:

<https://afni.github.io/qc-demo-repo/>

Reynolds, R. C., Taylor, P. A., & Glen, D. R. (2023). Quality control practices in fMRI analysis: Philosophy, methods and examples using AFNI. *Frontiers in Neuroscience*, 16, 1073800. <https://doi.org/10.3389/fnins.2022.1073800>

Paul A. Taylor, Daniel R. Glen, Gang Chen, Robert W. Cox, Taylor Hanayik, Chris Rorden, Dylan M. Nielson, Justin K. Rajendra, Richard C. Reynolds; A Set of fMRI Quality Control Tools in AFNI: Systematic, in-depth, and interactive QC with afni_proc.py and more. *Imaging Neuroscience* 2024; 2 1–39. doi: https://doi.org/10.1162/imag_a_00246